Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 04-Apr-2026

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1. Administrative & Study Identification

| Field | Data Entry / Details | | :--- | :--- | | **Full Study Title** | A Study to Compare the Combination of BMS-986504 With Pembrolizumab and Chemotherapy Versus Placebo Plus Pembrolizumab and Chemotherapy in First-line Metastatic Non-small Cell Lung Cancer Participants With Homozygous MTAP Deletion | | **Short Title/Acronym** | MountainTAP-29 | | **Protocol Number** | CA240-0029 | | **NCT Number** | NCT07063745 | | **Posting ID** | 986141587226771563 | | **Trial Status** | RECRUITING | | **Sponsor/Company Name** | Bristol Myers Squibb | | **Investigational Product** | BMS-986504 (MTA-cooperative inhibitor of PRMT5), Pembrolizumab, and Chemotherapy (Cisplatin [Trade Name: Platinol, ATC Code: L01XA01], Carboplatin, Pemetrexed, Paclitaxel, Nab-paclitaxel) | | **Phase of Development** | Phase 2 / Phase 3 | | **Medical Monitor** | Lei Deng (Investigator) / Bristol Myers Squibb |

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2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To compare the clinical benefit of the combination of BMS-986504 plus pembrolizumab and chemotherapy versus placebo plus pembrolizumab and chemotherapy in first-line metastatic non-small cell lung cancer participants with homozygous MTAP deletion. * **Secondary Objectives:** To evaluate safety, overall survival (OS), progression-free survival (PFS), objective response (OR), disease control, and time to objective response (TTOR) of the experimental combination.

2.2 Background & Rationale Methylthioadenosine phosphorylase (MTAP) gene deletion occurs in a significant portion of non-small cell lung cancers (up to 27%). When MTAP is deleted, MTA accumulates and binds to the PRMT5 enzyme, which is crucial for cancer cell survival. BMS-986504 is designed to selectively target and inhibit PRMT5 when it is bound to MTA (the PRMT5-MTA complex), leading to targeted cancer cell death. This study aims to determine if adding BMS-986504 to the standard-of-care first-line therapy (pembrolizumab and chemotherapy) yields superior clinical outcomes for patients with this specific genetic profile.

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3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Placebo-controlled (Placebo + Pembrolizumab + Chemotherapy) * **Blinding:** Double-blind * **Randomization:** Randomized

3.2 Planned Enrollment & Duration * **Target N:** 590 participants * **Number of Sites:** 102 locations globally * **Estimated Study Start:** 02-Jan-2026 * **Estimated Primary Completion:** 12-Aug-2031

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Metastatic (Stage IV or recurrent) non-small cell lung cancer (NSCLC) (as defined by the American Joint Committee on Cancer, Ninth Edition) with no prior systemic anti-cancer therapy for metastatic disease. 2. Histologically confirmed diagnosis of NSCLC and homozygous methylthioadenosine phosphorylase (MTAP) deletion or MTAP loss. 3. Eastern Cooperative Oncology Group (ECOG) performance status of 0-1. 4. At least 1 measurable lesion as per RECIST v1.1.

4.2 Exclusion Criteria 1. Nonsquamous participants must not have documented targetable oncogenic mutation or actionable genetic alterations (AGAs) for which there is a standard of care (SoC) available as first-line (1L) therapy. 2. Symptomatic brain metastases or spinal cord compression. 3. Any prior systemic therapy (chemotherapy, immunotherapy, targeted therapy, or biological therapy) for metastatic non-small cell lung cancer (mNSCLC). Note: One cycle of SoC treatment prior to randomization will be allowed for participants who require immediate treatment if clinically indicated. 4. Known or suspected impairment of gastrointestinal function that may prohibit the ability to absorb or swallow an oral medication without chewing or crushing. 5. Other protocol-defined Inclusion/Exclusion criteria apply.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** Administered in standard oncological cycles. * **Route of Administration:** Oral (BMS-986504 or Placebo) combined with Intravenous infusions (Pembrolizumab and investigator's choice of Chemotherapy). * **Duration of Treatment:** Until disease progression, unacceptable toxicity, or maximum allowed treatment cycles are reached.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive care and symptom management. * **Prohibited:** Concurrent administration of any other investigational agents or non-protocol systemic anti-cancer therapies.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, MTAP Status Testing, Medical History, Baseline Imaging | | **Baseline** | Day 0 | Randomization, First Dose Administration | | **Interim** | Per Cycle (e.g., Q3W) | Safety Labs, Treatment Dosing,

Efficacy Assessment (RECIST v1.1) | | **End of Study** | At Progression | Final Vital Signs, Exit Interview, IP Accountability, Survival Follow-up |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * 1. Progression-free survival (PFS) * 2. Overall Survival (OS): Phase 3 * **Measurement Method:** Central/ Local Laboratory Imaging Analysis (RECIST v1.1) and survival tracking.

7.2 Secondary & Exploratory Endpoints * Objective response (OR) (confirmed complete response (CR) or partial response (PR)) * Disease control (best overall response (BOR) of confirmed CR, confirmed PR, or stable disease (SD)) * Time to objective response (TTOR) (CR or PR) * Incidence and severity of treatment-emergent adverse events

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8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Continuous monitoring of participant safety via physical exams and laboratory testing at each treatment cycle.
- **Serious Adverse Events (SAEs):** Standard oncology reporting timeline, typically within 24 hours of investigator awareness.
- **Data Monitoring Committee (DMC):** Independent committee to periodically review aggregate safety and efficacy data.

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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) for primary efficacy evaluation; Safety Set for AE/SAE incidence tracking.
- **Sample Size Justification:** $N=590$, appropriately powered to detect a statistically significant difference in progression-free or overall survival between the experimental combination and the control arm.
- **Handling of Missing Data:** Standard right-censoring methods for time-to-event efficacy analyses (e.g., OS and PFS) for patients lost to follow-up.

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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

- **Monitoring Plan:** Regular onsite visits combined with centralized data monitoring to ensure protocol compliance and data integrity.
- **Audit Trail:** eCRF locking, source data verification (SDV), and query resolution processes implemented across all 102 locations.

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11. Study Identifiers & Governance

Primary ID: {{CA240-0029}} **Registry Number:** {{NCT07063745}} **Posting ID:** {{986141587226771563}} **Sponsor/Lead Organization:** {{Bristol Myers Squibb}} **Study Title:** {{MountainTAP-29}} **Official Regulatory Title:** {{A Randomized Phase 2/3 Study of BMS-986504 in Combination With Pembrolizumab and Chemotherapy Versus Placebo Plus Pembrolizumab and Chemotherapy in First-line Metastatic Non-small Cell Lung Cancer Participants With Homozygous MTAP Deletion}}

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12. Clinical Framework (Oncology Specific)

Primary Condition / Disease Area: {{Metastatic Non-small Cell Lung Cancer With MTAP Deletion}} **Preferred UMLS Name:** {{Metastatic Non-Small Cell Lung Carcinoma with MTAP Deletion}} **Semantic Type:** {{Neoplastic Process}} **Disease Definition:** {{A heterogeneous aggregate of at least three distinct histological types of lung cancer, including SQUAMOUS CELL CARCINOMA; ADENOCARCINOMA; and LARGE CELL CARCINOMA. They are dealt with collectively because of their shared treatment strategy.}} **MeSH Code:** {{D002289}} **HPO Code:** {{HP:0030358}} **SNOMED ID:** {{254637007}} **SNOMED Hierarchy:** {{448993007\|Carcinoma of lung\|Malignant epithelial neoplasm of lung (disorder); 39607008\|Lung structure\|Lung structure (body structure); 128632008\|Non-small cell carcinoma\|Non-small cell carcinoma (morphologic abnormality); 64572001\|Disease\|Disease (disorder)}} **Diagnosis Threshold:** {{Homozygous MTAP deletion or MTAP loss}} **Medical Methodology:** {{PRMT5-MTA cooperative inhibition combined with PD-1 checkpoint blockade and Chemotherapy}} **Optimization Target:** {{Targeted cancer cell death and tumor shrinkage}}

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13. Study Procedures & Methodology

Management Pathway: {{First-line systemic anti-cancer therapy pathway}} **Education Protocol (HCP):** {{Investigator meetings and site initiation training on protocol procedures}} **Education Protocol (Patient):** {{Informed consent process and oral dosing compliance training}} **Quality Audit Mechanism:** {{Routine clinical site monitoring and electronic data capture (EDC) auditing}}

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14. Observation & Follow-Up Schedule

Total Duration: {{Approximately 68 months (Estimated Completion August 2031)}} **Onsite Visit Cadence:** {{Every treatment cycle (e.g., Q3W/Q4W)}} **Remote Monitoring Frequency:** {{Continuous via central EDC and imaging platforms}} **Checklist Review Interval:** {{Prior to the start of each new treatment cycle}}

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	Lei Deng	____	**04-Apr-2026**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]